Inpatient Daily Review

Patient / ID _____ Second ID _____ Date/time _____

Bed _____ Hospital day _____ Service / clinician _____

Acuity Stable / Seriously ill / Critically ill Code status / verified _____

Key allergies / reactions _____

Precautions / isolation _____ Reason _____

Hospital course and interval events

Interval events / concerns; discussion, patient wishes / understanding; discrepancies.

Daily review BP _____ HR ____ RR ____ T ____ SpO2 ____ O2 device / flow _____

Consciousness _____ Intake / output _____ Time _____ Review / monitoring _____

Key results and trends Source/date; critical results: notify, act, review

Metric and unit	Date/time _____	Date/time _____	Date/time _____

Daily medication and device review

Current drugs: need / safety / response; home reconciliation.
Antibiotic review; line / catheter need; changes / rationale.

Focused examination and systems review

Changes / key positives and negatives.
General / heart / lungs / abdomen / skin / limbs / neuro.

Problem based assessment and plan

Today's goal and priorities _____

A: priority / status / likely cause; evidence for / against; key alternative / uncertainty.
P: results / consults; tests / treatment; expected response / monitoring / review; if-then below.

Daily safety and supportive care

Nutrition / swallow; pain / sleep; bowel / bladder;
mobility / rehab; VTE / bleeding;
falls / skin / delirium.

Discharge planning

Criteria / destination / care / access

Medicines / follow-up / understanding

Open tasks after discharge: owner / contact / deadline

Pending results/actions As of _____ Review and act before ticking; carry open items forward

Done	Pending result or action	Owner / contact	Due / next step
[ ]			
[ ]			
[ ]			
[ ]			

Contingency plan and handoff

Problem / if _____ Then / action _____

Responsible / supervising clinician + contact _____ Review by _____ Care limits _____

Receiver / time _____ Receiver confirmation / open actions _____

Reference frameworks: China's 18 core medical quality and safety systems; US CMS hospital Conditions of Participation (CoPs), Joint Commission National Patient Safety Goals (NPSGs; replaced by NPGs for hospitals in 2026), and ACGME Common Program Requirements (CPR); UK NICE CG50 and NHS England/CPOC NatSSIPs; Germany's G-BA QM-RL.
Drawing on selected principles from these frameworks, this template supports initial assessment, inpatient care, and the learning and teaching of clinical reasoning and patient management. It does not replace official hospital or institutional records, orders, management systems or governance responsibilities.

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Inpatient Background and Ongoing Needs

Retain and date changes. Reprint daily page; use linked continuation sheets as needed.

Patient / ID _____ Second ID _____ Date/time _____

Admitted _____ Service / team _____ Updated _____

Relevant medical history

Diagnoses / complications; specialists;
prior tests / treatment response.

Procedures and major events

Operations / admissions / ED visits; injuries;
devices / support therapies; complications.

Home medication history

Prescription / OTC / supplements; details / actual use / adherence;
source / pharmacy; discrepancies. Daily changes on page 1.

Allergies and reactions

Agent / reaction / severity.
None known / unknown / unassessed.

Social and family context

Home / support / access; family history; work / exposures;
substances; sexual / reproductive history as relevant.

Baseline function and mental status

Daily activities / mobility / aids / falls; cognition / mood;
communication / safety.

Goals and decision making

Priorities / capacity / surrogate; advance directives.
Current code status and care limits on daily page.

Contacts and care resources

Caregiver / PCP / specialists / pharmacy; interpreter;
home services / equipment / communication preferences.

Ongoing care needs and records

Baseline nutrition / swallow / oxygen / other support; rehabilitation / home services / access; missing records.

Dated background updates

Date / source / update / clinician informed; patient or caregiver factual check / understanding / corrections.

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